

=====

METROPOLITAN GENERAL HOSPITAL
DEPARTMENT OF INTERNAL MEDICINE & EMERGENCY
PATIENT MEDICAL EXAMINATION REPORT

=====

PATIENT DEMOGRAPHICS

=====

Patient Name: Ayan Ahmed Patient ID: PT-894021-X
Age / Gender: 34 Yrs / Male DOB: 12/14/1989
Admission Date: 10/14/2025 - 08:30 AM Discharge: 10/16/2025 - 04:00 PM
Attending Physician: Dr. Anas MRN: 8472-910-A
Department: Acute Care & Diagnostics Room / Bed: 304-B

=====

1. CHIEF COMPLAINT & PRESENT ILLNESS

=====

Chief Complaint: High-grade fever, chills, severe malaise, and generalized myalgia for 4 days.

History of Present Illness (HPI):

The patient is a 34-year-old male presenting with a 4-day history of acute onset fever peaking at 103.2°F (39.5°C). The fever is accompanied by rigor, diaphoresis, throbbing frontal headache, and retro-orbital pain. The patient reports persistent generalized body aches, muscle fatigue, and mild non-productive cough.

No history of recent international travel, though patient reports a hiking trip in wooded areas 10 days prior. Denies chest pain, shortness of breath, dysuria, abdominal pain, or skin rash prior to admission. Over-the-counter Acetaminophen (500mg) provided transient relief.

=====

2. VITAL SIGNS (UPON ADMISSION)

=====

- Temperature: 102.8 °F (39.3 °C) [Elevated]
- Heart Rate (Pulse): 110 bpm [Tachycardic]
- Blood Pressure: 118/76 mmHg [Normal]
- Respiratory Rate: 20 breaths/min [Slightly elevated]
- Oxygen Saturation: 98% on ambient air [Normal]
- Body Mass Index (BMI): 24.5 kg/m² [Normal]

=====

=====

3. PHYSICAL EXAMINATION

=====

=====

- General: Conscious, alert, oriented x3. Appears acutely ill, febrile, and diaphoretic.
- HEENT: Normocephalic. Mild conjunctival injection present bilateral. No scleral icterus. Pharynx mildly erythematous; no tonsillar exudates. Posterior cervical lymph nodes palpable, mobile, mildly tender (<1 cm).
- Cardiovascular: Tachycardic, regular rhythm. Normal S1 and S2. No murmur, rub, or gallop.
- Pulmonary: Clear to auscultation bilaterally. No rales, rhonchi, or wheezing.
- Abdomen: Soft, non-tender, non-distended. Normal bowel sounds. No hepatosplenomegaly.
- Musculoskeletal: Generalized tenderness in bilateral calf and thigh muscles. No joint swelling.
- Neurological: Cranial nerves II-XII intact. No neck stiffness or meningeal signs (Kernig/Brudzinski negative).
- Skin: Mild transient macular erythema over trunk; no petechiae or eschar noted.

=====

=====

4. LABORATORY & DIAGNOSTIC RESULTS

=====

=====

[COMPLETE BLOOD COUNT (CBC)]

Test Name	Result	Reference Range	Status

White Blood Cells (WBC)	13.8	4.5 - 11.0 x10 ³ /μL	HIGH
Red Blood Cells (RBC)	4.72	4.3 - 5.9 x10 ⁶ /μL	Normal
Hemoglobin (Hgb)	14.2	13.5 - 17.5 g/dL	Normal
Hematocrit (Hct)	42.1	41.0 - 50.0 %	Normal
Platelet Count	135	150 - 450 x10 ³ /μL	LOW
Neutrophils	82%	40 - 75 %	HIGH
Lymphocytes	12%	20 - 45 %	LOW

[INFLAMMATORY & BIOCHEMICAL MARKERS]

Test Name	Result	Reference Range	Status

C-Reactive Protein (CRP)	48.5	< 5.0 mg/L	HIGH
Erythrocyte Sed. Rate (ESR)	38	0 - 15 mm/hr	HIGH
Serum Glucose (Random)	104	70 - 140 mg/dL	Normal
Serum Creatinine	0.92	0.7 - 1.3 mg/dL	Normal
Blood Urea Nitrogen (BUN)	16	7 - 20 mg/dL	Normal
ALT (SGPT)	58	7 - 56 U/L	Mildly High
AST (SGOT)	62	8 - 48 U/L	Mildly High

[PATHOLOGY / INFECTION SCREENING]

- Dengue NS1 Antigen: Negative
- Malaria Smear (Thick/Thin): Negative for Plasmodium species
- COVID-19 / Influenza PCR: Negative
- Blood Cultures (2 sets): Pending (No growth at 24 hours)
- Urine Analysis: Clear, Specific Gravity 1.020, Leukocyte Esterase Negative

5. DIAGNOSTIC IMPRESSION / ASSESSMENT

1. Acute Febrile Illness (Pyrexia) secondary to suspected viral/bacterial infection.
2. Mild Leukocytosis with Left Shift and Elevated Inflammatory Markers (CRP, ESR).
3. Mild Thrombocytopenia (likely reactive/consumption due to acute infection).
4. Transient Mild Transaminitis.

6. TREATMENT & MANAGEMENT PLAN

1. Hydration: IV Fluids Normal Saline (0.9% NaCl) at 100 mL/hr to manage dehydration.
2. Antipyretics: Acetaminophen 1000 mg IV q6h as needed for temperature > 101°F.
3. Analgesics: Ibuprofen 400 mg PO q8h PRN for severe myalgia/headache.
4. Monitoring: Strict intake/output monitoring, vital signs checked q4h.
5. Antibiotic Prophylaxis: Empiric oral Doxycycline 100 mg BID initiated pending serology results (given tick exposure history).

7. DISCHARGE INSTRUCTIONS & FOLLOW-UP

- Patient afebrile for 24 hours prior to discharge. Vital signs stable.
- Continue oral hydration (2–3 liters of water daily).
- Complete 7-day course of prescribed oral antibiotic (Doxycycline 100mg BID).
- Follow up with primary care provider in 3-5 days for repeat CBC and LFTs.
- Seek immediate emergency care if fever recurs above 102°F, or if shortness of breath, severe abdominal pain, persistent vomiting, or skin rash develops.

Attending Physician Signature:

Dr. Anas

Board Certified in Internal Medicine

License No: #MD-992014-IL